

NHS – T2DR - Referral Form C

The Type Two Diabetes Pathway to Remission programme is an evidence-based intervention using Total Diet Replacement (TDR) to support people recently diagnosed with Type 2 diabetes to achieve significant weight loss and potentially attain diabetes remission (non-diabetic HbA1c results, at least 6 months apart, off all glucose-lowering medicines). There is no cost to participants with all TDR (shakes, soups and bars) funded by the NHS.

Eligibility Criteria:

- Aged 18 to 65 years (note that individuals aged over 65 years may be referred if clinically appropriate and if potential benefits for that individual are considered to outweigh potential risks associated with rapid weight loss [e.g. exacerbation of pre-existing frailty])
- Diagnosed with Type 2 diabetes within the last 6 years;
- A BMI of 27kg/m² or higher in people from White ethnic groups, adjusted to 25kg/m² or higher in people from Black, Asian and other ethnic groups.
- BMI obtained from self-measured weight by a Service User is acceptable for referral. If this cannot be obtained, a clinic-measured value within the last 12 months may be used, provided there is no concern from the referrer that the Service User's weight may have reduced since last measured such that the individual would not be eligible for the Service at present;
- A HbA1c measurement taken within the last 12 months, with values as follows:
 - If on diabetes medication, HbA1c 43 to 87 mmol/mol; or
 - If not on diabetes medication, HbA1c 48 to 87 mmol/mol;
 - provided there is no concern from the referrer that the Service User's HbA1c may have changed since last measured such that the individual would not be eligible for the Service at present; and
- Have attended for monitoring and diabetes review when this was last offered, including retinal screening, and commit to continue attending annual reviews, even if remission is achieved. (For avoidance of doubt, if a Service User is newly diagnosed then there is no requirement to wait for retinal screening to take place before offering referral)
- Individuals who meet any of the following exclusion criteria must not be referred to the Service and must not be accepted by the Provider. The Provider must confirm that the individual is eligible and so does not meet any of the following exclusion criteria prior to the individual's commencement of the intervention:

Individuals Must Not Be:

- Current insulin user;
- Pregnant or planning to become pregnant within the next 6 months;
- Currently breastfeeding;
- Discharged in the last 12 months from the NHS T2DR Programme after having commenced the programme (for clarity, this does not apply to people previously referred to the programme but who dropped out or decline prior to commencing the TDR Phase);
- Has at least one of the following significant co-morbidities;
 - active cancer;
 - heart attack or stroke in last 6 months;
 - severe heart failure (defined as New York Heart Association grade 3 or 4);
 - severe renal impairment (most recent eGFR less than 30mls/min/1.73m²);
 - active liver disease other than non-alcoholic fatty liver disease (NAFLD) (i.e. NAFLD is not an exclusion criterion);
 - active substance use disorder;
 - active eating disorder (including binge eating disorder);
 - porphyria; or
 - known proliferative retinopathy that has not been treated (this does not exclude individuals who are newly diagnosed and have not yet had the opportunity for retinal screening);
- Has had bariatric surgery; or
- Health professional assessment that the person is unable to understand or meet the demands of the NHS T2DR Programme and/or monitoring requirements (due to physical or psychological conditions or co-morbidities).

What is Low Calorie Diet, or Total Diet Replacement

- Total diet replacement refers to an approach where usual foods are replaced with a micronutrient-replete formulated diet
- The TDR products usually take the form of shakes or soups, however other product forms are available
- There are a number of commercial entities producing TDR products. The composition of such products is regulated by European legislation
- Generally, a product is consumed at each mealtime. Some TDR regimes also include an additional specially formulated 'snack'
- When used as intended and no other foods are consumed, the total daily calorie intake on the TDR phase of the NHS T2DR is around 800-900 kcals
- Though other approaches such as low calorie diets with 'real foods' may be effective for some people, the strongest evidence base in achieving Type 2 Diabetes remission through non-surgical means is currently with TDR approaches

Aims of the NHS T2DR Programme Pilot

- Reduction in weight of Service Users and maintenance of weight loss achieved
- Achievement of remission of Type 2 diabetes / reduced glycaemic parameters
- Reduction in medication usage

RESPONSIBILITIES – Referring GP Surgeries and T2DR Provider

Responsibilities of the referring GP practice:

- Identify eligible patients and offer referral as appropriate
- Provide information on concept of remission of Type 2 Diabetes, the T2DR service and potential risks and benefits to obtain informed consent
- Discuss and agree any relevant medication changes with the potential service user
- Make clear to the potential Service User that changes should only be enacted on the first day of starting the TDR intervention
- Clinical responsibility for an individual's medication changes remains with general practice at all times that the individual is associated with the service
- Respond to any clinical need to further adjust medications according to capillary blood glucose and blood pressure monitoring by the Provider
- Respond to adverse events if patient contacts practice directly with an urgent clinical need or is directed to the GP practice by the Provider
- Arrange review of patient at 6 months and 12 months after starting T2DR programme with repeat HbA1c –with further medication adjustment as necessary
- Confirm with the Service User that should they proceed on the NHS T2DR Programme, the Service User:
 - Agrees to continue attending yearly diabetes review appointments at their GP practice, regardless of whether remission is achieved;
 - Will contact their GP practice or urgent care service as appropriate if they have any unexpected or concerning symptoms which are considered urgent; and
 - Will notify their GP practice if they disengage or drop out before the end of their intervention.

Responsibilities of T2DR Provider:

- Attempt contact with patients referred within 5 working days to provide further information about the T2DR service and book Individual Assessment
- Respond to advice requests with appropriate advice within 5 Operational Days
- Be aware of whether any potential Service User has a medication change or not
- Ensure that the Provider and the Service User understand the specific medication changes which are required (or that no medication changes are required) and the Provider has received details of changes (or confirmation that no change is necessary) in writing from the referrer.
- Ensure the potential Service User is also aware of the medication changes proposed (or that no medication changes are required) although there is no requirement for this to be submitted to the Service User by the referrer in writing
- Ensure that, prior to the first day of TDR Phase, the Service User understands the specific medication changes which are required (or that no medication changes are required).
- To confirm that it and the Service User have been provided with information relating to medication changes prior to the commencement of the intervention
- Perform / arrange for monitoring of capillary blood glucose and blood pressure (in people taking BP-lowering medications at time of referral)
- Identify where capillary blood glucose and blood pressure fall outside of specified parameters and communicate appropriately with GP practice for further action

- confirm that no additional glucose-lowering or blood pressure-lowering medications have been started since medication changes were last agreed (including if the recommendation was for no medication changes to take place) and specified in writing to the Provider and communicated to the Service User)
- confirm with the Service User that they will not be taking sulphonylureas, meglitinides or SGLT2 inhibitors as of the first day of the TDR intervention
- Take steps to ensure that referrers can obtain advice regarding medication changes
- Ensure that communication in this regard is facilitated between referrers and the Provider's Medical Director (or another suitably experienced registered medical practitioner within the meaning of Schedule 1 of the Interpretation Act 1978 with an MRCP or MRCPGP)
- Act as initial contact for patients experiencing a concurrent or adverse event which is not considered an emergency
- Appropriately triage and respond to adverse events –including signposting the patient to the GP practice or to other services
- Provide a starter pack of fibre supplements and ongoing supply as necessary
- Optimise uptake and retention on the programme

If the Provider:

- Cannot confirm that the Provider and the Service User have been provided with confirmation from the referrer of the specific medication changes which are required (or that no medication changes are required); and/or
- cannot confirm that no additional glucose-lowering or blood pressure-lowering medications have been started since medication changes were last communicated by the referrer (including communication of no medication changes to be made); and/or
- cannot confirm with the Service User that they will not be taking sulphonylureas, meglitinides or SGLT2 inhibitors (if applicable) as of the first day of the TDR intervention,
- then the Provider must defer the Service User's TDR Phase start date and take such action as it necessary to ensure that the above matters are confirmed. Once the matters above have been confirmed, the Provider must promptly arrange the Service User's commencement of the TDR Phase

REQUIRED MEDICATION ADJUSTMENTS – PLEASE READ

Blood glucose-lowering medication adjustments:

- People on 1-2 glucose-lowering medications should stop these medications on the first day of TDR
- People on ≥ 3 medications should stay on metformin only (or, if not taking metformin as it is contraindicated / not tolerated, stay on an oral medication which is safe with TDR, e.g. DPP4 inhibitor or pioglitazone) and stop the remaining glucose-lowering medications on the first day of TDR
- Counsel the patient about the osmotic symptoms of diabetes and advise them of when and how to seek appropriate support

Blood pressure-lowering medication adjustments:

- Note that BP-lowering medications include those used for other indications (e.g. tamsulosin for benign prostatic hypertrophy, furosemide for oedema) as well as those used specifically for managing BP
- If blood pressure is considered uncontrolled at time of referral (systolic ≥ 140 mmHg or diastolic ≥ 90 mmHg), make no changes to BP-lowering medications
- If blood pressure is considered controlled at time of referral (both systolic < 140 mmHg and diastolic < 90 mmHg), one BP-lowering medications should be adjusted on the first day of TDR
- If reviewing the patient remotely, it is reasonable to use self-reported blood pressure. If not available, the last clinic-recorded blood pressure may be used, provided there is no concern of white coat hypertension or that blood pressure may have changed significantly since last measured
- Medications being used specifically and solely for managing blood pressure, in a particular patient, are the priority for adjustment. Suggested process:
 - Identify the medications used by the patient solely for managing blood pressure (i.e. not also being used for nephropathy, angina, heart failure, BPH, migraines etc)
 - Stop the medication which would have been added last according to current NICE guidance - unless other clinical factors affect decision making
 - If not being used for other indications, this would be (in order of stopping first):
 - Spironolactone or alpha-blocker or beta-blocker
 - Thiazide diuretic (or calcium-channel blocker)
 - Calcium-channel blocker (or thiazide diuretic)
 - ACE-inhibitor or Angiotensin receptor blocker
- If the patient is taking medications which affect blood pressure, but all are being used for other indications (none are being used solely to manage blood pressure):

○ use clinical judgement and shared decision making and take into account the BP reading ○ cautiously reduce the dose of this medication rather than stopping it ○ consider arranging early review, in relation to the specific indication for the medication ○ in some circumstances, it may be reasonable not to adjust these medications initially but to carefully monitor and respond accordingly					
• Counsel the patient about symptoms of postural hypotension and when and how to seek support					
Patient Details					
Title:	Title	Preferred contact number:			
First Name:	Given Name	Can we leave a voicemail?		Yes ☐	No ☐
Surname:	Surname	Alternative contact number:			
		Can we leave a voicemail?		Yes ☐	No ☐
Address:	Home Full Address (single line)				
Postcode:	Home Address Postcode	What is the patient's Ethnicity:		Ethnic Origin	
Email address:	Patient E-mail Address	What is the patient's first language		Main Language	
NHS number:	NHS Number	Does the patient speak English?		Yes ☐	No ☐
Date of birth:	Date of Birth	Is patient on Severe Mental Illness register?		Yes ☐	No ☐
Gender:	Gender(full)	Does the patient have any learning disability?		Yes ☐	No ☐
		Does the patient have any mobility issues?		Yes ☐	No ☐
		Does the patient have a previous eating disorder?		Yes ☐	No ☐
		Is the patient registered disabled?		Yes ☐	No ☐
		Date Patient Diagnosed with Diabetes:		Free Text Prompt	
Does patient have any Food Allergies, Intolerances and/or any Religious/Cultural considerations that need taking in to account? If yes, provide details below:				Yes ☐	No ☐
Notes/any other information?					
• Any agreed medication adjustments with the patient (to be enacted on first day of TDR) – must be specified in writing to the patient and T2DR provider (incl. if no changes are needed)					
• Has this been provided along with the referral? (email copy of letter is acceptable)				Yes ☐	No ☐
Referral Weight/Height					
Referral weight & date:	Single Code Entry: Body weight	Height:	Single Code Entry: Standing height	BMI:	Single Code Entry: Body mass index
Blood Test					
HbA1c	Single Code Entry: Haemoglobin A1c level - International Federation of Clinical Chemistry and Laboratory Medicine standardised				
Fasting Plasma Glucose	Single Code Entry: Plasma fasting glucose level				
Blood Pressure	Single Code Entry: O/E - blood pressure reading				
GP Details					
GP Practice name:	Organisation Name	GP Practice code:	Organisation National Practice Code		
GP Practice address:	Organisation Full Address (single line)				
GP Practice Tel number:	Organisation Telephone Number				
Referrer Details					
Referrer name & role:	Free Text Prompt Free Text Prompt				
Date of referral:	Short date letter merged				
Information to be shared with patient when a referral is made:					
• Their personal and medical information is being shared • The programme is delivered by Reed Wellbeing, and data will be shared securely between Reed Wellbeing, NHS England and the Local Authority for the purposes of delivering the programme effectively and monitoring programme performance					

- The purpose of sharing this information is for them to participate on the programme
- Their personal and medical information will be stored securely on electronic systems and not be used for any purpose other than programme delivery and monitoring
- The results of preventative diabetes screening will be shared securely with their GP
- Their information may be shared with NHS England for programme evaluation purposes only
- Their data will be treated as confidential and held, shared and disposed of in line with all legal requirements (including the General Data Protection Regulation) and NHS guidance (which includes the Caldicott Guidelines)
- Should they take up the programme, their data will be retained for the duration of the contract with NHS England.
- Should they not take up the programme, or cease to attend, their individual data will be retained for 3 years after they have been discharged back to the care of their GP practice.

Referrer to confirm they have discussed the programme with patient prior to making the referral. Expressing the commitment required and the patient has agreed to the referral and is willing to participate in Type Two Diabetes Pathway to Remission Programme.

Yes ☐

No ☐

Referrer to confirm that the patient is able to understand and meet the demands of the NHS T2DR programme and monitoring requirements.

Yes ☐

No ☐

Current Medication and Medication Changes

- **Please discuss this section with the patient and provide them with a copy of the agreed medication changes. We will not be able to proceed with the referral if they have not received this in writing.**
- **The guidance regarding required adjustments for blood-pressure lowering medications and blood-glucose lowering medications are included with this form.**
- Please provide details of blood glucose-lowering medications (i.e. treatments for diabetes) and Blood Pressure-lowering medications which are currently taken by the patient. Please specify the agreed changes to these medicines to occur on the first day of Total Diet Replacement (TDR), such as stating if the medication is to be stopped, specific changes to dosing / frequency, or if there is to be no change.
- **Sulfonylureas, meglitinides and SGLT2 inhibitors must be stopped to safely start TDR**
- Note that blood pressure-lowering medications includes medicines used for indications other than hypertension – i.e. diuretics, alpha blockers for BPH, beta blockers for migraine prophylaxis.
- **If the patient is taking any other medications which may need adjustment according to weight or dietary changes (e.g. warfarin), it is the responsibility of the referrer to ensure that processes are in place for these medicines to be safely adjusted. If any such medicines are being taken, referral should only be sent if, prior to referral, the referrer has established who will be responsible for obtaining weight readings (or other monitoring parameters - e.g., INR), the frequency of such checks, how this will be recorded, how the prescriber will be notified and how dose changes will be communicated to the patient.**
- Reed Wellbeing will encourage all patients to monitor their blood glucose and blood pressure at each engagement, with the GP practice notified of any need for further medication adjustments.

Patient Name: **Full Name** **Date:**

Medication changes to take place on 1st day of TDR:

Not on any blood-glucose medicines	☐		
Class of blood-glucose lowering medicines used at time of referral	Name of specific medication(s) used, if any, from this class	Dosage and frequency at time of referral	Agreed changes on 1st day of TDR (no change / specify dose change / stop)
<i>Biguanides (e.g. metformin)</i>			
<i>Sulfonylureas (e.g. gliclazide, glimepiride)</i>			
<i>Meglitinides (-glinides)</i>			
<i>Thiazolidinediones (e.g. pioglitazone)</i>			
<i>DPP4 inhibitor (-gliptins)</i>			
<i>SGLT2 inhibitors (-flozins)</i>			
<i>GLP-1 analogues (-tides)</i>			
<i>Alpha-glucosidase inhibitor (e.g. acarbose)</i>			
Not on any blood-pressure medicines	☐		
Blood-pressure lowering medicine	Dosage and frequency at time of referral	Agreed changes on 1st day of TDR (to be stopped / dose change / no change)	

Any other medicine needing adjustment due to weight or dietary changes	Agreed process for managing the monitoring and dose adjustment of these medicines (must be established prior to referral)	
Have these medication changes been discussed and agreed with the patient?		Yes ☐ No ☐
If any new glucose-lowering medications or blood pressure-lowering medications are commenced following referral but prior to start of TDR, this section on medication changes must be updated accordingly with revised versions provided to the patient and Reed Wellbeing – please send to T2DR.SSOT@nhs.net		
Past Relevant Medical History / Co-Morbidities / Any Additional Relevant Information		
Does the patient confirm that they will: - continue attending diabetes review appointments at their GP practice, regardless of whether remission is achieved - notify the GP practice of any unexpected or concerning symptoms which are considered urgent - notify their GP practice if they disengage or drop out before the end of the intervention		Yes ☐ No ☐
Once complete please send these forms via e-mail to: T2DR.SSOT@nhs.net Only send patient information via secure NHS email.		
If you are unsure how to do this or have any questions, please contact Reed Wellbeing on 0800 092 1191 or using the above email address		
Please code referral as 'Referral to total diet replacement programme' (SNOMED 1239571000000105)		